Just admitted.

A hospitalist's quick guide for patients & families

Someone you love has been admitted to the hospital. Everyone is talking fast, different people keep walking into the room, and you're trying to figure out what happens next.

Start here.

Part 1

Who's who?

Hospitalist

The doctor primarily managing your medical care while you're admitted. We coordinate the overall plan and often work with specialists involved in your care.

Bedside nurse

Your closest point of contact throughout the shift. Tell your nurse about new or worsening symptoms and ask them questions about medications, monitoring and the day's plan.

Specialist / consultant

A doctor asked to help with a specific problem—for example, cardiology, gastroenterology, nephrology or infectious disease. They may make recommendations, while the hospitalist continues coordinating the overall medical plan.

Primary care doctor

Your regular outpatient doctor may not personally manage your hospital stay, depending on the hospital. They become especially important again after discharge.

Case manager / social worker

They help with the practical side of leaving the hospital safely—such as home services, rehabilitation, equipment, insurance-related needs and other discharge arrangements.

Part 2

What are "rounds"?

Rounds are when your hospital team reviews what happened overnight, your labs and tests, examines you, and decides what needs to happen next.

Your hospitalist may also be caring for many other hospitalized patients, so you may not know exactly when they will arrive.

> **Hospitalist tip:** Keep your most important questions written down. If you aren't going to be at the bedside, ask your loved one and the bedside team how family communication can best be handled.

Part 3

The 5 questions to ask every day

You do not need to understand every medical term. Start with these.

1 *"What is the main problem you're most concerned about right now?"*

Early in a hospitalization, the final diagnosis may not be clear yet. That's okay. You're trying to understand what the team is most concerned about and what they are working to figure out.

2 *"What are we watching for today?"*

Ask what would tell the team that things are improving—and what changes would concern them.

3 *"What tests or procedures are planned, and what are you hoping they will tell us?"*

A useful test should answer a clinical question or help guide what happens next. Understanding that question can make the plan much easier to follow.

4 *"What needs to happen before I can safely go home?"*

Ask this early. The answer might include improving oxygen needs, kidney function, pain control, mobility, eating and drinking, completing a procedure, arranging medications, or creating a safe discharge plan. Think of discharge as a checklist—not simply a date on the calendar.

5 *"What changes should I tell the nurse about right away?"*

If something feels newly or significantly different, tell the bedside team. Examples include new or worsening chest pain, difficulty breathing, fainting, significant confusion, new weakness, severe or rapidly worsening pain, or another sudden change that concerns you.

You do not need to diagnose the problem yourself. A useful phrase is:

"I'm concerned because this is new, or a change from earlier."

Part 4

What happens overnight?

The doctor caring for you overnight may not be the same hospitalist you saw during the day. That is normal.

The overnight team focuses heavily on keeping patients safe, evaluating new or worsening problems, and responding when something changes.

Some non-urgent decisions may appropriately wait for the daytime team that knows the longer-term plan.

But don't wait for morning.

If something feels significantly different or worse, don't wait until morning just because it is nighttime.

Tell your nurse.

Part 5

Before you leave the hospital

Before discharge, make sure you understand:

- [ ] What diagnosis or problems were treated?
- [ ] Which medications should I take now?
- [ ] Which home medications were stopped, started or changed—and are those changes temporary or permanent?
- [ ] Do I need follow-up blood work, imaging or other testing?
- [ ] Who do I need to follow up with, and when?
- [ ] What symptoms mean I should call my clinician, seek urgent evaluation, or return for emergency care?
- [ ] Who should I contact if I have questions after discharge?

One last hospitalist tip

Before you walk out the door, compare the medication list you came in with to the medication list you're leaving with.

If something changed and you don't understand why, **ask before you leave.**

✂ Screenshot this page, or print it and keep it by the bed

Save these 5 questions

1. What is the main problem you're most concerned about right now?
2. What are we watching for today?
3. What tests or procedures are planned, and what are you hoping they will tell us?
4. What needs to happen before I can safely go home?
5. What changes should I tell the nurse about right away?

Today's notes

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Follow Dr. Ijay for practical medicine, real hospital stories, and the journey behind the white coat.

This guide provides general educational information and is not medical advice for your individual situation. Your bedside healthcare team knows your specific medical history and current condition. If you are concerned about a new or worsening symptom while hospitalized, tell your bedside team promptly.